CLINICAL OBS & GYNAE

1. **Most common cause of secondary post-partum hemorrhage:**
    a) Atony
    b) Placental tissue
    c) Trauma
    d) Endometritis (uterine infection)
2. **primary post-Partum hemorrhage occurs within:**
    a) 1 hours
    b) 6 hours
    c) 12 hours
    d) 24 hours
3. **common cause of perinatal mortality worldwide**
    a) congenital malformation
    b) Prematurity
    c) Asphyxia
    d) Infection
4. **female come with abdominal pain on US show cyst with echogenicity like dermoid cyst The cause of pain**
    a) rupture
    b) torsion
    c) infection
    d) hemorrhage
5. **to diagnose ectopic pregnancy**
    a) Serum level of B hCG
    b) transvaginal US
    c) abdominal US with fluid in DOP (Douglas pouch)
    d) Laparoscopy
6. **In breech presentation**
    a) increase with parity
    b) pass of meconium indicates fetal distress

c) breech extraction flexed presentation
d) cord prolapse more with flexed

7. postmenopausal bleeding evaluation:

a) vaginal U S
b) D&C
c) hysteroscopy and endometrium biopsy
d) abdominal U/S

8. Commonest cause of induction of labor

a) Pregnancy induce hypertension
b) prolong pregnancy
c) IUGR
d) IUFD

9. Assessment of Fetal wellbeing:

a) Doppler umbilical artery
b) Doppler umbilical vein
c) Middle cerebral artery
d) fetal heart rate by pinard

10. pregnant in 37 weeks came with vaginal bleeding and abdominal pain most likely due to

a) Placental abruption + fetal heart rate 160
b) placenta previa
c) vasa previa + fetal heart rate 60

11. pregnant lady admitted to hospital because has high blood pressure she develops tonic Colonic convulsion and nurse call you what is your management

a) A.mg sulphate
b) airway, breathing, circulation (ABC)
c) IV line
d) labetalol

12. Common cause of primary Post partum hemorrhage

a) Atony

b) Placental tissue
c) Trauma
d) endometritis

13. **35 years old grand multipara has had a major postpartum hemorrhage (PPH) following normal delivery, mechanical and pharmacological measure have failed to control the bleeding, examination has confirmed that there are no retained placental tissue in uterine cavity and absence of trauma to genital tract, what is the most appropriate first line surgical management?**
    a) Balloon Tamponade
    b) ligation of internal iliac artery
    c) postpartum hysterectomy
    d) lynch or modified compression suture.
14. **D.E.is a 19 years old college student who presented to the ER for vaginal spotting. She reported that she had a positive home pregnancy test 2 days ago, her last menstrual period was 5 weeks ago. The ER physician ordered a quantitative human chorionic gonadotropin (HCG) test that was reported as 213 m IU/ml. what is the source of this hormone?**
    a) Fetal pituitary gland
    b) Developing trophoblast
    c) Ovarian corpus luteum
15. **You are attending the labor of patient who has prolonged 2nd stage of labor. you note the fetal head start to retract after being tightly applied to the vulva, what is the next most appropriate management step?**
    a) Apply fundal pressure
    b) C-section
    c) All fours maneuver
    d) MC-Robert's maneuver
16. **pregnant lady has hyperthyroidism on carbimazole developed sore throat investigation should be done**
    a) FBC
    b) TFT
    c) Throat swab
    d) CRP
17. **24-year-old lady delivered by C/S Section in day 7 she developed SOB on climb stairs, what is causing her condition**
    a) PE
    b) Amniotic fluid embolism

c) Hypertrophic cardiomyopathy
d) Congestive heart failure

18. **48-year-old lady presented with heavy menstrual bleeding and intermenstrual bleeding her HB is 112% and US showed no endometrial abnormality next step**
    a) coagulation screening
    b) Diagnostic hysteroscopy
    c) endometrial sampling
    d) pelvic MRI
19. **31-year-old woman presents in preterm labor at 34 weeks of gestation, her labor progresses quickly and she delivers baby boy. Both mother and baby appear to be in good health, the woman requested delayed cord clamping. What time frame would be recommended for delayed cord clamping in this situation?**
    a) Clamp cord as soon as possible
    b) 30 second - 3 minute
    c) 10-30 second
    d) 4-6 minutes
20. **19 years old female presented with amenorrhea for 5-week pregnancy test was positive and HCG level 7000, uterus empty on U/S how to treat her**
    a) Conservative
    b) Medical treatment with methotrexate
    c) Misoprostol oral
    d) Surgery
21. **Mid wife examines pregnant lady in 21 week of her gestation and she found her has fused labium minora, what is the type of FGM in this lady?**
    a) Type 1
    b) Type 2
    c) Type 3
    d) Type 4
22. **Study of VBAC in woman had previous vaginal delivery after one C/S section, type of study is?**
    a) Cohort study
    b) cross sectional
    c) Survey
    d) Case control

23. **Pregnant lady in her 38 weeks of gestation, presented with labor pain and on PV there is palpable nasal bridge and palpebral fissure, what is diameter of this presentation?**

    a) Submentobregmatic
    b) Mentovertical
    c) Suboccipitobregmatic
    d) Occipitofrontal

24. **Pregnant lady in 28 week of her gestation, high vaginal swab for B. streptococci is positive. what you will do?**

    a) Give her course of oral antibiotics
    b) Intrapartum injectable antibiotics
    c) If not treated her baby will get severe neonatal sepsis
    d) Reassurance

25. **32-year-old primigravida in her 36 weeks of gestation presented with generalized tonic clonic convulsion, her BP 150/100, urine contain ++ protein. What is the initial step in management?**

    a) Diazepam
    b) Mg sulphate
    c) Hydralazine
    d) Nifedipine

26. **The daily requirement of calcium supplementation in pregnant and lactating women:**

    a) 600 mg/day
    b) 800 mg/day
    c) 1200 mg/day
    d) 1500 mg/day

27. **female came to your clinic has hirsutism and acne, her BMI 37, which means she is obese and has DM, what is the Pathophysiology of PCO:**

    a) conversion of estrogen into androgen
    b) hyperinsulinemia
    c) increase level of FSH
    d) hyperandrogenism

28. **Pt come with recurrent abortion in 3 months, diagnosed as Antiphospholipid antibody syndrome, what treatment can help to complete pregnancy?**

a) Aspirin and Heparin
b) Cortisol and Antibody
c) Immunoglobulin
d) Antibiotics

29. Single lady for one year, has undergoes to un-protective sexual intercourse 6 days ago, her 1st day of the cycle was 15 days ago, now she came searching for Emergency contraception:

a) copper IUD
b) Ulipristal
c) it's too late to give contraception

30. Mechanism of action of Tamoxifen:

a) selective estrogen receptive modulator
b) aromatase inhibitor
c) progesterone receptor antagonist

31. Head station assessment relative to:

a) ischial spine
b) ischial tuberosity
c) pelvic outlet
d) all of the above

32. amount of oxytocin used in cs

a) 5 IU
b) 10 IU
c) 15 IU
d) 40 IU

33. massive bleeding is

a) loss 1 volume in 24 h
b) loss 1 volume in 2 h
c) loss 50% in 24 h
d) loss 50% in 2 h

34. amount of fresh frozen plasma used in massive bleeding

a) 2 unit
b) 4 unit
c) 6 unit
d) 10 - 15 ml/kg

35. in which period ulipristal contraceptive is effective

a) 24 h
b) 72h
c) 120h
d) 48 h

36. side effect of warfarin in pregnancy:

a) nasal hypoplasia
b) renal agenesis
c) intracranial hemorrhage

37. about bacterial vaginosis

a) Answer diagnosed by hay\Ison criteria
b) Caused by actinomyces Israeli
c) Odorless vaginal discharge
d) Amsel criteria have no role in diagnosis

38. after how long we consider 3rd stage of labor is abnormal

a) 20 min
b) 30 min
c) 60 min
d) 90 min

39. vaginal tear include rectal mucosa is

a) 1st degree
b) 2nd degree
c) 3rd degree
d) 4th degree

40. in suspected PPROM in speculum examination if no liquor appear next step is

a) send her home
b) repeat test after 4h c-pad test
c) PBL test

41. prim gravida in 40 weeks with spontaneous rupture membrane and 2 contractions per 10 min your action

a) give oxytocin
b) cs

c) leave her

42. prim gravida with cervix 2 cm and intact membrane your action is

a) cs
b) check after 4 h
c) give oxytocin

43. pregnant lady with twin and sudden rupture of membrane your action

a) cs
b) vaginal examination to check liquor
c) vaginal exam to check cord prolapse

44. about using partogram

45. amount of body iron in hemoglobin

a) 50%
b) 60%
c) 75%
d) 90%

46. female with primary amenorrhea and absent uterus and vagina cause is

a) gonadal agenesis
b) Mullerian agenesis
c) PCOS

47. female with 28 days cycle omit her contraceptive in day 17 and 18 now she is on day 20 she worries about being pregnant your advice

a) reassuring that nothing will happen
b) start new vial of contraceptive
c) take the 2 missing pill as fast as possible and continue in her contraceptive
d) it is too late

48. multipara underwent previous cs then has successful VBAC after two years now pregnant after 9 years from the last pregnancy with fetus with wt. 3500g fetus position. factor that suggests she will now have successful vaginal delivery is:

a) previous VBAC
b) fetal size
c) period between pregnancy
d) fetal position

49. there are 7 stages of normal labor, the 6th stage is:

a) extension
b) flexion
c) internal rotation
d) external rotation

50. all of the following are contraindications to use instrumental delivery except:

a) fetal malpresentation
b) fetal distress
c) maternal pelvic fracture
d) cervical dilatation 10 cm
e) active maternal infection

51. In shoulder dystocia your action:

a) MC Robert and fundal pushing
b) MC Robert and axial traction

52. Which of the following findings would confirm a diagnosis of 'severe' pre-eclampsia?

a) Serum creatinine of 0.8 mg/dL
b) Serum AST (aspartate transaminase) 140 mg/dL
c) 1+ proteinuria on dipstick
d) BP 158/96 mmHg
e) Uric acid of 5.8 mg/Dl

53. Which of the following tests is the most accurate in diagnosing a pulmonary embolus during pregnancy?

a) Ventilation-perfusion (V/O) scan
b) Spiral chest CT scan
c) EKG
d) Pulmonary angiography
e) Serum D-dimer measurements

54. At what gestational age is the amniotic fluid volume at its maximum?

a) 22 weeks
b) 26 weeks
c) 30 weeks
d) 34 weeks

55. In monozygous twin pregnancy, what type of chorionicity results when the blastocyst divides between days 9 and 12 post-conception?

a) Conjoined twins
b) Dichorionic/diamniotic placentation

c) Monochorionic/monoamniotic placentation
d) Monochorionic/diamniotic placentation

56. Which of the following statements regarding cesarean section delivery is TRUE?

a) Maternal morbidity is significantly increased with cesarean compared with vaginal delivery
b) Elective cesarean section delivery can be performed before 39 weeks' gestation without documenting fetal lung maturity
c) Elective surgeries (such as myomectomy) can be performed safely at the time of cesarean section
d) A desire for permanent sterilization by bilateral tubal ligation is an acceptable indication for cesarean section delivery
e) One-third of all cesarean section deliveries in the USA are elective cesarean sections

57. 46-year-old, non-pregnant, morbidly obese woman presents with irregular periods over the past 6 months. Which of the following represents the most appropriate initial management in this setting?

a) Oral contraceptives
b) Endometrial ablation
c) office hysteroscopy
d) Dilation and curettage (D&C)
e) Endometrial biopsy

58. A 23-year-old woman with confirmed pelvic inflammatory disease (PID) presents with right upper quadrant pain. What is the MOST likely etiology?

a) Cholecystitis
b) Nephrolithiasis
c) Perihepatic abscess
d) Asherman syndrome
e) Fitz-Hugh-Curtis syndrome

59. Which of the following is the MOST accurate statement about spontaneous miscarriage?

a) Most frequently occurs in second trimester
b) Chromosomal anomalies are rare
c) Happens to more than half of all conceptions
d) Young maternal age is a risk factor
e) Triples the likelihood of miscarriage at next pregnancy

60. patient presents with vulvar irritation and pruritis. Speculum examination reveals a hyperemic, edematous, vaginal vault with odorless discharge. The pH is 4.0. What is the MOST likely diagnosis?

a) Bacterial vaginosis (BV)
b) Candidiasis
c) Trichomoniasis
d) Gonococcal vaginitis
e) Hidradenitis suppurativa

61. What is the MOST proven cause of recurrent pregnancy loss (RPL)?

a) Luteal phase deficiency
b) Listeria monocytogenes exposure
c) Uterine anomalies
d) Parental chromosomal abnormalities
e) Environmental toxins

62. Which of the following non-malignant adnexal masses is derived from all three germ cell layers?

a) Ovarian fibroma
b) Para tubal cyst
c) Hydrosalpinx
d) Endometrioma
e) Mature cystic teratoma

63. 16-year-old girl presents with a fever (39°C), nausea, pelvic pain, and generalized malaise a few days after unprotected intercourse. What is the MOST likely diagnosis?

a) Trichomoniasis
b) Primary herpes simplex infection
c) Syphilis
d) Bartholin gland abscess
e) Lymphogranuloma venereum

64. According to menstrual bleeding abnormality, heavy irregular intermenstrual bleeding is

a) Menometrorrhagia
b) Menorrhagia
c) Metrorrhagia
d) Dysmenorrhea

65. Endocervical duct closure due to metaplastic change:

a) Bartholin cyst
b) Gartner cyst

c) follicular cyst
d) Nabothian duct

66. 16 years old individual who is phenotypically a female presented with primary amenorrhea, The karyotype was XY, what is the diagnosis?

a) Complete Androgen insensitivity syndrome
b) 5 alpha reductase deficiency
c) Kallman syndrome
d) Turner syndrome

67. Common causes of antepartum hemorrhage (APH, bleeding from the genital tract from 24 weeks' gestation) include which of the following?

a) Vasa previa
b) Undetermined origin
c) Placenta previa
d) Placental abruption

68. In Shoulder dystocia:

a) Due to obstructing outlet
b) Can occur with normal birth wt.
c) In 1 in 200
d) Preventable in majority of cases

69. What is the single greatest cause of intrauterine growth restriction (IUGR) worldwide?

a) Maternal cocaine use
b) Malaria
c) Uteroplacental insufficiency
d) Foetal chromosomal abnormality
e) Foetal alcohol syndrome

70. A 39-year-old G2P1 presents at 38 weeks' gestation with a BP 174/112 mmHg and 2+ proteinuria. You proceed with induction of labour. Your primary aim in lowering her BP is to prevent which of the following complications?

a) Seizures (eclampsia)
b) Renal failure
c) Pulmonary oedema
d) Preterm labour
e) Stroke

71. A 39-year-old G2P1 presents at 18 weeks' gestation for her scheduled foetal anatomic survey. During the examination, the foetal anatomy is noted to be normal, but the cervical length is measured at 15 mm. Despite counselling

about the lack of consistent data showing a benefit in this setting, the patient requests that a cervical cerclage be placed. Which of the following, if present, would be considered an absolute contraindication to cervical cerclage placement?

a) Advanced maternal age
b) Prolapse of foetal membranes into the vagina
c) Positive foetal fibronectin test
d) Rupture of foetal membranes
e) Placenta previa

72. A number of screening tests are performed in all pregnancies at designated gestational ages. Which of the following screening tests is correctly matched with the gestational age?

a) Glucose load test at the first prenatal visit
b) Chlamydia test at 22-24 weeks
c) foetal anatomic survey at 34-36 weeks
d) Group B ß-haemolytic streptococcus (GBS) carrier status at 35-37 weeks
e) Rubella serology at the 6-week postpartum visit

73. patient presents for her first prenatal visit at 10 weeks' gestation. Her routine prenatal intake laboratory tests reveal that she is not immune to rubella. what should you recommend?

a) Administer MMR vaccination now
b) Administer MMR vaccination at 28 weeks
c) Administer MMR vaccination postpartum
d) She does not need MMR vaccination

74. Regarding Amniotic fluid embolism

a) If she survives in the first 30 min, she will develop DIC, pulmonary oedema and respiratory distress syndrome

75. Which of the following maternal cardiac lesions is associated with a pregnancy-associated maternal mortality rate of 50% or more?

a) Surgically corrected tetralogy of Fallot
b) Ventricular septal defect (VSD)
c) Marfan syndrome with an aortic root diameter of 5 cm
d) Previous myocardial infarction
e) Prosthetic heart valve

76. Which of the following obstetric emergencies is associated with a maternal mortality rate of 90%?

a) Myocardial infarction
b) Pulmonary embolism
c) Postpartum haemorrhage
d) Eclampsia
e) Amniotic fluid embolism

77. What is the primary goal of diagnosing and treating gestational diabetes?

a) To prevent congenital malformations
b) To prevent miscarriage
c) To decrease the incidence of foetal macrosomia
d) To reduce the risk of type 2 diabetes in the offspring
e) All of the above

﴿وَمَا تَوْفِيقِي إِلَّا بِاللَّهِ ۚ عَلَيْهِ تَوَكَّلْتُ وَإِلَيْهِ أُنِيبُ﴾